



Role of Patient-Reported Measures (PRMs) in Addressing Ethical Challenges During Telemedicine Consultation: A Narrative Review

Asiya Attar

Kasturi Shukla

Follow this and additional works at: <https://pxjournal.org/journal>



Part of the [Business Administration, Management, and Operations Commons](#), [Health and Medical Administration Commons](#), [Medical Education Commons](#), [Public Health Commons](#), and the [Social and Behavioral Sciences Commons](#)



RESEARCH

Role of Patient-Reported Measures (PRMs) in Addressing Ethical Challenges during Telemedicine Consultation: A Narrative Review

Asiya Attar ^a, Kasturi Shukla ^{b,*}

^a Junior Research Fellow, Symbiosis Institute of Health Sciences, Symbiosis International (Deemed University), Pune, India

^b Associate Professor, Symbiosis Institute of Health Sciences, Symbiosis International (Deemed University), Pune, India

ABSTRACT

Introduction: Telemedicine has revolutionized medical care delivery by emphasizing on Patient-Centred Care (PCC). Patient Reported Measures (PRMs) can assist in providing PCC since it assesses the treatment outcomes and patient experience. However, using PRMs is not just limited to gauging clinical efficiency; it can also play a role in addressing the ethical challenges in telemedicine. Yet this aspect of PRMs remains underexplored. **Method:** Literature search was conducted using a combination of keywords from the Google Scholar, Scopus, and Web of Science databases. 126 peer-reviewed journal articles in English from 2015 to 2024 were screened, and 34 relevant full-text studies were reviewed. **Discussion:** While telemedicine provides access to healthcare beyond the physical boundaries of traditional facilities, ethical issues like privacy, data security, and informed consent still exist. PRMs can help address these challenges, thereby improving PCC. This can help to make telemedicine consultations more ethical and equitable. **Conclusion:** This article emphasizes how PRMs can facilitate ethical telemedicine by providing informed consent, safeguarding data privacy, and ensuring equitable patient participation. The suggested conceptual framework applies PRMs at every phase of the teleconsultation process, ensuring a systematic route toward the facilitation of ethical standards and PCC in e-health.

Keywords: Ethics, Patient reported measures, PRMs, Patient reported outcome measures, PROMs, Patient reported experience measures, PREMs, Telemedicine

1. Introduction

In recent years, e-health technologies have become valuable tools in healthcare since they provide creative ways to improve patient education, communication, and self-management.^{1–3} Eysenbach defines e-health as “a new discipline that combines medical informatics, public health, and health, as well as services and information related to health that are provided or improved via the use of the internet and other technologies.”⁴ E-health includes a wide range of digital health services, such as online health information resources, mobile health apps, electronic health records, and telemedicine.^{3,5,6}

Telemedicine is defined as “the use of telecommunications technologies to provide medical information and services.”^{7,8} Following the COVID-19 pandemic, telemedicine has experienced an unprecedented surge in its use.^{9–11} As a result, many organisations have integrated telemedicine into standard care settings to expand healthcare access.^{12,13} It is crucial to enhance Patient Satisfaction (PS) and Patient Experience (PE) during teleconsultation.^{9,10,14} Hence, the need to provide better patient-centred care in telemedicine is being increasingly recognised by governments and healthcare organisations.¹⁵

Received 12 May 2025; revised 21 July 2025; accepted 29 July 2025.
Available online 7 August 2025

* Corresponding author.
E-mail address: kasturishukla18@gmail.com (K. Shukla).

<https://doi.org/10.35680/2372-0247.2032>

2372-0247/© The Author(s), 2025. Published in association with The Beryl Institute. This work is licensed under a Creative Commons Attribution-Noncommercial-No Derivative Works 4.0 License.

Patient-Centred Care (PCC) is defined as “providing care that is respectful of and responsive to individual patient preferences, needs, and values, and ensuring that patient values guide all clinical decisions.”¹⁶ PCC is better executed when health systems and practitioners employ Patient-Reported Measures (PRMs).^{17,18} PRMs capture and assess health behaviours, PS, PE, and Health-Related Quality of Life (HRQoL) directly from the patient.^{19,20} Therefore, PRMs are essential for quality improvement initiatives since they offer the patient’s perspective on both high-quality and low-quality healthcare aspects.¹⁷ This data is directly acquired from patients using PRM scales such as Patient-Reported Outcome Measures (PROMs) and Patient-Reported Experience Measures (PREMs).^{17,20,21}

Patient-Reported Outcome Measures (PROMs) are “questionnaires measuring the patients’ views of their health status.”²² Patient-Reported Experience Measures (PREMs) are “questionnaires measuring the patients’ perceptions of their experience whilst receiving care (NABH 2020).” These scales are the essential aspects for evaluating the quality of a healthcare system.^{23–27} However, these scales are not limited to measuring HRQoL and can assist with ethical challenges arising in physical and digital healthcare settings.¹⁹ Yet, existing studies primarily focus on clinical outcomes or technological effectiveness. Additionally, there is a lack of comprehensive research on how these technologies can facilitate ethical decision-making, encourage patient autonomy, and guarantee equitable care across various demographics.

To address this gap, a Research Question (RQ) was formed:

RQ. How can PRM scales be effectively used in telemedicine to identify and mitigate ethical challenges while improving the quality and patient-centeredness of virtual care?

The authors aimed to conduct a narrative review by exploring how PRMs can be integrated into telemedicine to address ethical challenges in virtual healthcare delivery. Moreover, the authors have also proposed a framework to employ PRM scales to ensure ethical telemedicine practice.

Specifically, the objectives of the study were to:

- To highlight the key ethical challenges during a telemedicine consultation

- To explore how PRMs (PROMs & PREMs) are used in telemedicine
- To understand the role of PRMs in addressing the key ethical challenges during a telemedicine consultation

1.1. Search strategy

A combination of keywords like “Patient-Reported Measures,” “PRMs,” “PROMs,” “PREMs,” “ethics,” and “telemedicine” was used to search for relevant literature. Google Scholar, Scopus, and Web of Science were used to identify suitable articles. Articles from peer-reviewed journals in the English language published from 2015–2024 were included. After screening the titles and abstracts of 126 articles, 34 full texts of relevant articles were reviewed.

2. Narrative synthesis of findings

2.1. Ethical conundrums in telemedicine

With the growth of healthcare organizations, healthcare delivery is no longer limited by institutional boundaries, nationality, or geography. Apart from clinical settings, telemedicine offers services in homes, schools, libraries, and other public and governmental locations.^{28–30} These changes are driven by the belief that people would want to remain in comfortable, familiar surroundings and have greater control over their health.^{28,29} Although telemedicine services have many acknowledged benefits, there are a number of ethical concerns that should be taken into account when using this service.³¹

Conflicts between several facets of technology use and ethical norms, such as abridgement of privacy (including merging and mining data), erroneous and outdated data, and security breaches, have given rise to ethical issues with telemedicine.^{28,32,33,35,36} Additionally, linking a patient’s medical and personal data to achieve interoperability for individual records, personal health management, and public health is another design-related ethical issue. Online information quality and accuracy are also major concerns.^{33–35,37} Table 1 illustrates the key ethical conundrums in telemedicine.^{38–44}

Furthermore, from an ethical point of view, telemedicine needs to be significantly improved, and certain guidelines and codes of conduct must be properly implemented. Here, PRM tools could prove to be beneficial in collecting data that would be useful in making the guidelines. In order to

Table 1. Key ethical conundrums in Telemedicine.

Key Ethical Challenges Identified	Telemedicine Perception	Patient Perspective
Informed consent	Obtaining informed consent can be less comprehensive in virtual settings, where patients might click through agreements without full understanding	Patients may feel they were not properly informed about the procedure or platform use, affecting their autonomy
Privacy	Telemedicine platforms often store or transmit sensitive health information over digital networks, which may be vulnerable to breaches	Patients may fear their personal and medical information could be exposed or misused, leading to mistrust in the system
Data security	Telemedicine systems may store data on third-party servers, and breaches can occur due to weak cybersecurity or inadequate data governance policies	Patients may fear their health data could be stolen, leaked, or monetized without consent
Quality of care	Concerns exist that virtual consultations may lack the depth of physical examinations or emotional connection compared to in-person care	Patients may feel that virtual care is rushed or impersonal, questioning the thoroughness of diagnosis or advice
Malpractice/professional accountability	The virtual setting may tempt some providers to cut corners, overbook appointments, or lack engagement due to absence of physical oversight	Patients may perceive providers as distracted, less committed, or delivering lower-quality care online

(Source: Author's Creation).

understand this, we need to first understand the use of PRMs in telemedicine.

2.2. Understanding the use of PRM tools in telemedicine

In recent years, a growing number of studies have assessed telemedicine services using PRMs.⁴⁵ The use of digital platforms allows the collection of real-time data via PRMs and monitors the progress of patients from a distance. The integration of PRMs into telemedicine represents a significant stride in delivering PCC since it provides accessibility of care, communication among the patient-care team, and clinician attitudes and behaviour during virtual care delivery (Fig. 1).⁴⁶ Additionally, PRMs (PROMs and

PREMs) can address special challenges and opportunities so that healthcare providers can offer thorough and responsive care.

The usage of PROMs and PREMs has grown in conjunction with the development of telemedicine applications. Therefore, these tools help to identify practical factors (such as organisational, relational, and environmental) that could either enhance or impede the quality, safety, and accessibility of care.⁴⁷⁻⁴⁹ In the context of telemedicine, PRMs can assist in identifying the factors, such as the degree of interoperability among various providers (healthcare facilities, professionals, and technology suppliers). Additionally, the ease of use of a particular technology and the actual functioning of telemedicine

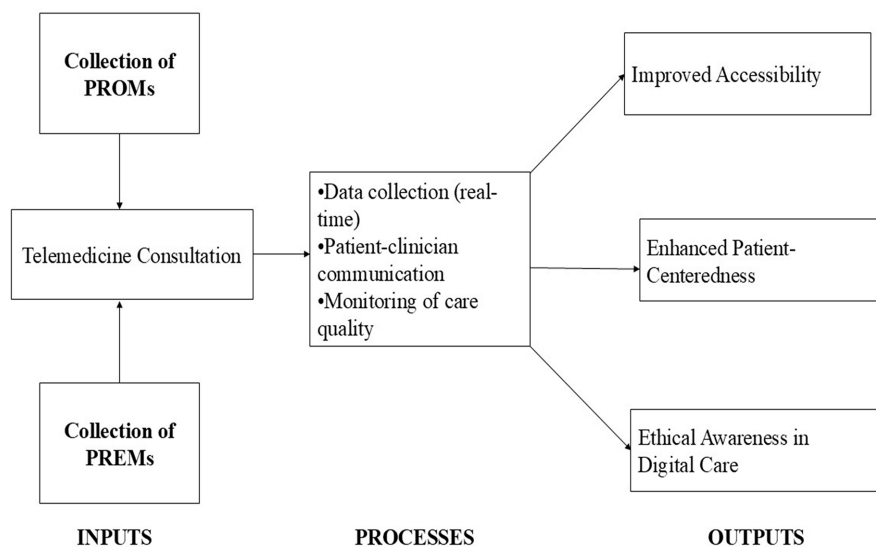


Fig. 1. Use of PRM tools in Telemedicine.
(Source: Author's Creation).

Table 2. Role of PRM tools in ethical issues in Telemedicine.

Ethical Issues in Telemedicine	Role of PRM tools
Informed Consent	PRM tools bring clarity and an understanding of the consultation process. The tools can track patient consent and ensure that it is informed and documented
Privacy & Confidentiality,	PRM tools help to secure the data of patients, ensuring compliance with privacy regulations and transparency regarding data use and sharing
Data Protection	The PRM tools will ensure the security, confidentiality, and integrity of patients' data through the use of encryption, safe storage, and access controls while empowering patients to keep track of and control how their data is shared or used
Quality of Care	PRM tools evaluate and track patient experiences and outcomes, allowing providers the chance to offer the most tailored, responsive care
Malpractice and Professional Integrity	The quality of care and PS are monitored by PRM tools, providing valuable data to evaluate professional conduct and accountability in virtual care

(Source: Author's Creation).

in settings such slow internet connections can also be highlighted.⁵⁰ Since virtual care is a nascent stage of care provision, the use of PRM can be important to ensure that the services provided are grounded in patient-centeredness.⁵¹ In addition to these advantages, the implementation of PRMs can be used to address ethical issues during telemedicine sessions.⁵²

2.3. Role of PRM tools in ethical challenges during telemedicine consultations

Digital health technologies aim to lessen disparities in the way patients receive or experience medical treatment. However, the use of these technologies may have a reversed effect, exacerbating inequality either directly or indirectly.⁵² Here, PRMs can track and evaluate the impact of digital health, which would help to clarify whether differences in experience with care are relevant. The PRM tools can collect data and provide the outcome, PS, and PE to highlight the ethical conundrums with the use of telemedicine. Table 2 illustrates the role of PRM tools in ethical challenges related to telemedicine.

Based on a synthesis of the literature reviewed, the authors have proposed a framework for integrating PRMs to ensure ethical and patient-focused telemedicine consultations.

3. Proposed framework to employ PRM scales to ensure ethical telemedicine practice

A structured framework integrating PRMs can be critical for ensuring ethical telemedicine practice. Considering the prominent ethical issues (Table 1) and the applicability of PRM tools (Fig. 1), the authors have proposed a framework to ensure an eth-

ical telemedicine consultation session. Each ethical concern is coupled with a corresponding role of the PRM tools so that a holistic patient-centred telemedicine consultation can be achieved. Using Table 2 as the basis for this framework, Fig. 2 illustrates the flow of a telemedicine consultation session.

This framework illustrates the possible use of PRM tools to address ethical challenges that frequently arise in the course of telemedicine consultations.

Step 1: Start by obtaining informed consent forms and implementing PREMs to ensure that patients are well-versed in the details of any consultation that includes risks and benefits.

Example: “Did the telemedicine platform explain to you the purpose of this consultation and the associated risks in a way you could understand?”

Step 2: As the consultation progresses, the framework brings in the sensitive aspect of data protection and privacy through both PROMs and PREMs. While PROMs can be used to understand how secure patients feel about their health data, PREMs can capture the overall experience with the system’s privacy measures.

Example of PROMs: “Are you worried that your health information might be accessed by someone who is not authorized?”

Example of PREMs: “Was it clear from the telemedicine platform how your personal and medical information would be protected?”

Step 3: Additionally, PROMs can also enable patients to report their symptoms and outcomes directly while gathering real-time health data, assisting clinicians in making well-informed decisions.

Example: “Have you noticed any changes in your symptoms since your last visit?”

Step 4: Before the end of the session, PREMs can be implemented to record the communication and

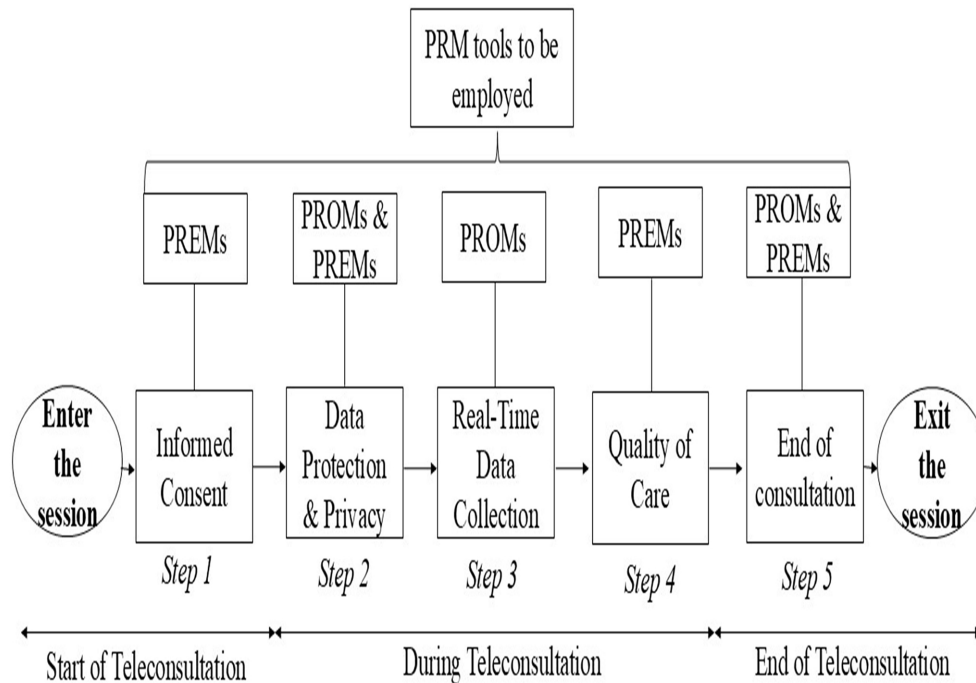


Fig. 2. Proposed framework for the implementation of PRM tools during a Telemedicine consultation session. (Source: Author's Creation).

experience of the consultation in order to comprehend the quality of care and PCC delivered.

Example: i) “Did the health care provider explain your treatment choices clearly?”

ii) “Were your questions and concerns adequately addressed during the consultation?”

Step 5: Once a session has been completed, employing PROMs and PREMs can help in reporting a patient’s progress in terms of their health condition and allow valuable feedback regarding the experience with the telemedicine process.

Example of PROMs: “Have your symptoms improved, worsened, or stayed the same since the visit?”

Example of PREMs: “Was the telemedicine consultation helpful in addressing your main health concern?”

The data obtained by using this framework can be used to enhance the ethical delivery of telemedicine consultation. Therefore, by employing PROMs and PREMs at each step, the framework addresses all the major ethical issues in telemedicine consultation and delivers better PCC.

4. Limitations

Although this study provides insights into the use of PRMs and highlights the key ethical issues, it also has

a few limitations. Firstly, the search was restricted to English-language peer-reviewed publications only, and therefore, studies published in other languages could have been overlooked. Secondly, the study does not include an exhaustive search of all possible evidence on this subject area. Gray literature, like policy briefs, white papers, and unpublished research, was not included, which might have resulted in a loss of important practical insights or new evidence from non-academic sources.

5. Conclusion

The PRMs have been a transformative integration within the telemedicine aspect since it evaluates PCC within a digital environment. Using PRMs adds more insights into individual needs of healthcare, opens effective communication paths, and tailors personalized treatment regimens. However, beyond these traditional applications, PRMs can be used to potentially overcome the ethical challenges during a telemedicine consultation. Thus, this review firstly identifies some of the fundamental ethical issues, such as data confidentiality and protection, informed consent, quality of care, and professional responsibility. Secondly, this study demonstrates how PRMs can help identify and solve these issues. Hence, with the ongoing growth of telemedicine, incorporating PRMs

as part of standard practice could prove to be beneficial in maintaining quality, trust, and accountability within virtual health systems.

Additionally, using the synthesis from this study, the authors have proposed a framework for the integration of PRMs into telemedicine consultation. This framework presents an applied path towards improving the ethical standards and PCC in online care provision.

For future research, longitudinal studies can be conducted that could help determine the real-world effect of incorporating PRMs on patient outcomes, trust, and system-level accountability. Additionally, context-specific PRM tools developed specifically for telemedicine could amplify their efficacy in facilitating ethical virtual care.

Acknowledgements

The authors would like to acknowledge Symbiosis International (Deemed) University, Pune, India.

Funding Acknowledgements

This research received no specific grant from any funding agency in the public, commercial, or not-for-profit sectors.

Conflict of interest

The authors declare that there are no conflicts of interest.

References

- Da Fonseca MH, Kovalski F, Picinin CT, Pedrosa B, Rubbo P. E-Health practices and technologies: A systematic review from 2014 to 2019. *Healthcare (Basel)*. 2021;9(9):1192.
- Stoumpos AI, Kitsios F, Talias MA. Digital transformation in healthcare: Technology acceptance and its applications. *Int J Environ Res Public Health*. 2023;20(4):3407.
- Seguro JM, Santos MP. Opportunities and challenges for E-health in Hip and Knee Arthroplasty - a summary of recent literature review. *Patient Experience Journal*. 2025;12(1):137–145.
- Eysenbach G. What is e-health?. *Journal of Medical Internet Research*. 2001;3(2):e833.
- Milliren CE, Lindsay B, Biernat L, Smith TA, Weaver B. Can digital engagement improve outcomes for total joint replacements? *Digital Health*. 2022;8:20552076221095322.
- Sridhar S, Hunter AM, McCrory B. Digital patient engagement at a perioperative surgical home implemented community hospital. *Patient Experience Journal*. 2023;10(1):141–154.
- Perednia DA, Allen A. Telemedicine technology and clinical applications. *JAMA*. 1995;273(6):483.
- Wootton R, Craig J, Patterson V. Introduction to telemedicine. *CRC Press*; 2017. <https://shorturl.at/8QG6X>.
- D'Anza B, Pronovost PJ. Digital health: Unlocking value in a post-pandemic world. *Population Health Management*. 2022;25(1):11–22.
- El-Sherif DM, Abouzid M, Elzarif MT, Ahmed AA, Albakri A, Alshehri MM. Telehealth and Artificial Intelligence insights into healthcare during the COVID-19 pandemic. *Healthcare*. 2022;10(2),385.
- Watson L, Link C, Qi S, Photitai E, Chmielewski L, Fode D, Delure A. Understanding patient experiences before and during the COVID-19 pandemic: A quasi-experimental comparison of in-person and virtual cancer care. *Patient Experience Journal*. 2022;9(1):95–102.
- Cynthia J, Tim R. Transformations in health information technology and the impact on patient experience. *Patient Experience Journal*. 2019;6(2):5–8.
- Snoswell CL, Caffery LJ, Haydon HM, Thomas EE, Smith AC. Telehealth uptake in general practice as a result of the coronavirus (COVID-19) pandemic. *Australian Health Review*. 2020;44(5):737–40.
- Guan X, Porter MC, Omodt PG. Patient experience diagnosis: Using telemed simulation to assess health care provider verbal and nonverbal communication issues to prescribe potential interventions. *Patient Experience Journal*. 2024;11(1):53–64.
- Mercadal-Orfila G, Herrera-Pérez S, Piqué N, Mateu-Amengual F, Ventayol-Bosch P, Maestre-Fullana MA, Fernández-Cortés F, Barceló-Sansó F, Rios S. Implementing systematic patient-reported measures for chronic conditions through the naveta value-based telemedicine initiative: observational retrospective multicenter study. *JMIR mHealth and uHealth*. 2024;12(1):e56196.
- IoM. Committee on Quality of Health Care in America. Crossing the quality chasm: A new health system for the 21st century. National Academies Press; 2001. <https://www.ncbi.nlm.nih.gov/books/NBK22274/>.
- Tzelepis F, Sanson-Fisher RW, Zucca AC, Fradgley EA. Measuring the quality of patient-centered care: Why patient-reported measures are critical to reliable assessment. *Patient Preference and Adherence*. 2015;831–5.
- Wasim A, Sajjan M, Majid U. Patient-centered care frameworks, models and approaches: An environmental scan. *Patient Experience Journal*. 2023;10(2):14–22.
- Fung CH, Hays RD. Prospects and challenges in using patient-reported outcomes in clinical practice. *Quality of Life Research*. 2008;17:1297–302.
- Le Corroller AG, Bonastre J. Patient-reported measures: How useful in health economics?. *The European Journal of Health Economics*. 2023;24(1):1–4.
- Fink R. Pain assessment: The cornerstone to optimal pain management. *Baylor University Medical Center Proceedings*. 2000;13(3):236–239.
- National Accreditation Board for Hospitals and Healthcare Providers (NABH). 5th edition. 2020. <https://healthcarearchitecture.in/wp-content/uploads/2022/01/NABH-5-STD-April-2020.pdf>.
- Dawson J, Doll H, Fitzpatrick R, Jenkinson C, Carr AJ. The routine use of patient-reported outcome measures in healthcare settings. *BMJ*. 2010;340.
- Nelson EC, Eftimovska E, Lind C, Hager A, Wasson JH, Lindblad S. Patient-reported outcome measures in practice. *BMJ*. 2015;350.
- Stover AM, Haverman L, van Oers HA, Greenhalgh J, Potter CM. ISOQOL PROMs/PREMs in Clinical Practice

- Implementation Science Work Group Sara Ahmed Joanne Greenhalgh Elizabeth Gibbons Lotte Haverman Kimberly Manalili Caroline Potter Natasha Roberts Maria Santana Angela M. Stover Hedy van Oers. Using an implementation science approach to implement and evaluate patient-reported outcome measures (PROM) initiatives in routine care settings. *Quality of Life Research*. 2021;30:3015-33.
26. Attar A, Shukla K. Listening to the unsaid: Utilizing patient-reported outcome measures (PROMs) to manage the dental anxiety of a special child. *Patient Experience Journal*. 2024;11(2):9-12.
 27. Shukla K, Attar A. The role of PROMs in mitigating infectious disease crises: A healthy ageing perspective. Sustainable Ageing: Reimagining Healthy Ageing Through the Lens of Flagship Campaigns. *Cham: Springer Nature Switzerland*. 2024:189-209.
 28. Kaplan, B., Litewka, S. Ethical challenges of telemedicine and telehealth. *Cambridge Quarterly of Healthcare Ethics*, 2008;17(4):401-416.
 29. Keenan AJ, Tsourtos G, Tieman J. The value of applying ethical principles in telehealth practices: Systematic review. *Journal of Medical Internet Research*. 2021;23(3):e25698.
 30. Hull SC, Oen-Hsiao JM, Spatz ES. Practical and ethical considerations in telehealth: Pitfalls and opportunities. *The Yale Journal of Biology and Medicine*. 2022;95(3):367.
 31. Nittari G, Khuman R, Baldoni S, Pallotta G, Battineni G, Sirignano A, Amenta F, Ricci G. Telemedicine practice: Review of the current ethical and legal challenges. *Telemedicine and e-Health*. 2020;26(12):1427-37.
 32. Layman EJ. Ethical issues and the electronic health record. *The Health Care Manager*. 2008;27(2):165-76.
 33. Bauer KA. Home-based telemedicine: A survey of ethical issues. *Cambridge Quarterly of Healthcare Ethics*. 2001;10(2):137-46.
 34. Nykanen P. E-health systems: Their use and visions for the future. E-Health Systems Diffusion and Use: The Innovation, the User and the Use IT Model. *IGI Global*. 2006;281-293.
 35. Sari PK, Handayani PW, Hidayanto AN, Yazid S, Aji RF. Information security behavior in health information systems: A review of research trends and antecedent factors. *Healthcare*. 2022;10(12):2531.
 36. Saxena V, Shende N. Teledentistry: Paradigm shift in health care. *Shineeks Publishers*. 2023. <https://shorturl.at/TgItf>.
 37. Bardaweel SK, AlMuhaissen SA, Alkurdi NH, Tayyem HH. Data privacy and confidentiality from the perspectives of general public and health care providers in Jordan. *International Journal of Clinical Practice*. 2021;75(6):e14117.
 38. Keenan AJ, Tsourtos G, Tieman J. Promise and peril-defining ethical telehealth practice from the clinician and patient perspective: A qualitative study. *Digital Health*. 2022;8:20552076211070394.
 39. Bobb MR, Van Heukelom PG, Faine BA, Ahmed A, Messerly JT, Bell G, Harland KK, Simon C, Mohr NM. Telemedicine provides noninferior research informed consent for remote study enrollment: A randomized controlled trial. *Academic Emergency Medicine*. 2016;23(7):759-65.
 40. Andreadis K, Muellers KA, Lin JJ, Mkuu R, Horowitz CR, Kaushal R, Ancker JS. Navigating privacy and security in telemedicine for primary care. *American Journal of Managed Care*. 2024;30.
 41. Houser SH, Flite CA, Foster SL. Privacy and security risk factors related to telehealth services—a systematic review. *Perspectives in Health Information Management*. 2023;20(1):1f.
 42. Baughman DJ, Jabbarpour Y, Westfall JM, Jetty A, Zain A, Baughman K, Pollak B, Waheed A. Comparison of quality performance measures for patients receiving in-person vs telemedicine primary care in a large integrated health system. *JAMA Network Open*. 2022;5(9):e2233267.
 43. Verma N, Buch B, Taralekar R, Acharya S. Diagnostic concordance of telemedicine as compared with face-to-face care in primary health care clinics in rural India: randomized crossover trial. *JMIR Formative Research*. 2023;7:e42775.
 44. Fogel AL, Kvedar JC. Reported cases of medical malpractice in direct-to-consumer telemedicine. *JAMA*. 2019;321(13):1309-10.
 45. Knapp A, Harst L, Hager S, Schmitt J, Scheibe M. Use of patient-reported outcome measures and patient-reported experience measures within evaluation studies of telemedicine applications: Systematic review. *Journal of Medical Internet Research*. 2021;23(11):e30042.
 46. Bajgain B, Rabi S, Ahmed S, Kiryanova V, Fairie P, Santana MJ. Patient-reported experiences and outcomes of virtual care during COVID-19: A systematic review. *Journal of Patient-Reported Outcomes*. 2023;7(1):126.
 47. Corazza I, Gilmore KJ, Menegazzo F, Abols V. Benchmarking experience to improve paediatric healthcare: Listening to the voices of families from two European children's University Hospitals. *BMC Health Services Research*. 2021;21:1-3.
 48. Gilmore KJ, Corazza I, Coletta L, Allin S. The uses of patient reported experience measures in health systems: A systematic narrative review. *Health Policy*. 2023;128:1-0.
 49. Pennestri F, Banfi G. The experience of patients in chronic care management: Applications in health technology assessment (HTA) and value for public health. *International Journal of Environmental Research and Public Health*. 2022;19(16):9868.
 50. Barger S, Castellini G, Vitale JA, Guida S, Banfi G, Gianola S, Pennestri F. Effectiveness of telemedicine for musculoskeletal disorders: Umbrella review. *Journal of Medical Internet Research*. 2024;26:e50090.
 51. Lloyd H, Wheat H, Horrell J, Sugavanam T, Fosh B, Valderas JM, Close J. Patient-reported measures for person-centered coordinated care: A comparative domain map and web-based compendium for supporting policy development and implementation. *Journal of Medical Internet Research*. 2018;20(2):e54.
 52. Roxburgh N. Will older adults be represented in patient-reported data? Opportunities and realities. *Bioethics*. 2024;38(9):763-9.